

HOSPITAL DISCHARGE SUMMARY

PATIENT DEMOGRAPHICS

Name: Patient 29

Age: 70 years old

Sex: Male

MRN: 000-TEST-29

Admission Date: [Current admission date]

Discharge Date: [Current discharge date]

Length of Stay: 3 days

PRIMARY DIAGNOSIS

Type 2 Diabetes Mellitus with hyperglycemic episode

SECONDARY DIAGNOSES

- Gout, chronic tophaceous
- Benign prostatic hyperplasia (BPH) with nocturia
- Hypertension, controlled
- Hyperlipidemia

HOSPITAL COURSE

Patient 29 presented to Metro Medical Center via ED with chief complaint of polyuria, polydipsia, and malaise × 5 days. Initial labs revealed admission glucose 347 mg/dL, HbA1c 8.9%, consistent with suboptimal glycemic control and acute decompensation. Urinalysis negative for ketones; no evidence of DKA or HHS. Patient had discontinued home metformin 2 wks prior per his report due to GI upset.

Course was uncomplicated. Endocrinology consulted on day 1. Insulin was initiated for glycemic control. Patient educated on carbohydrate counting, meal timing, and foot inspection technique. Repeat glucose values trended down to 180–220 mg/dL by discharge. No acute complications during admission. Gout flare managed with colchicine PRN. BPH stable on

current regimen.

MEDICATIONS AT DISCHARGE

| Medication | Dose | Frequency | Duration |

|---|---|---|---|

| Insulin glargine | 20 units | q.d. at bedtime | ongoing |

| Insulin lispro | 10 units | with meals (q.i.d.) | ongoing |

| Metformin | 500 mg | BID with breakfast & dinner | ongoing |

| Lisinopril | 10 mg | q.d. | ongoing |

| Atorvastatin | 40 mg | q.d. at bedtime | ongoing |

| Allopurinol | 300 mg | q.d. | ongoing |

| Tamsulosin | 0.4 mg | q.d. at bedtime | ongoing |

| Colchicine | 0.6 mg | PRN for acute gout (max 3 tabs/day) | as needed |

GLUCOSE MONITORING & HOME REGIMEN

- Home glucose monitoring BEFORE meals & at bedtime using finger-stick glucometer
- Target glucose range: 120–180 mg/dL fasting; <200 mg/dL postprandial
- Log readings & bring to all follow-up visits
- Review insulin technique with visiting nurse on [date in 3 days]

ACTIVITY & WEIGHT-BEARING STATUS

Full activity as tolerated. No restrictions.

DIETARY RECOMMENDATIONS

- Diabetic diet, consistent carbohydrate, low sodium (<2 g/day)
- Avoid sugary beverages & processed foods
- Maintain adequate hydration
- Nutritionist referral for outpatient medical nutrition therapy

FOOT CARE INSTRUCTIONS

- Inspect feet daily for cuts, blisters, or sores
- Wash feet daily with lukewarm water; dry thoroughly between toes
- Moisturize with lotion (avoid interdigital spaces)
- Wear well-fitting shoes; avoid going barefoot
- Annual diabetic foot exam with podiatry

FOLLOW-UP APPOINTMENTS

1. Endocrinology clinic — in 2 weeks with Dr. [name]. Bring glucose log.
2. Primary Care — in 1 week to review discharge paperwork & adjust medications as needed
3. Podiatry — in 4 weeks for preventive foot exam

RED FLAG SYMPTOMS — SEEK IMMEDIATE CARE FOR: